

Name: Daniel S May | DOB: 10/11/1989 | MRN: 10105654130 | PCP: Joshua W Clark, NP | Legal Name:
Daniel S May

Office Visit - Aug 15, 2025

with Stacey Gray, MD at Mass Eye and Ear Sinus Center



Notes from Care Team



Progress Notes by Stacey Gray, MD at 8/15/2025 9:00 AM

Massachusetts Eye and Ear Infirmary

Department of Otolaryngology

Sinus Center

Stacey T Gray, MD

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Boston, MA 02114

Telephone: 617-573-4188

Pt name: Daniel S May

Date: 8/15/2025

MEEI #: 8026121

History Present Illness:

Mr. Daniel May is a 35 y.o. commercial real estate broker who presents with history of chronic rhinosinusitis. He presented to the Emergency Department in July 2025 with symptoms of congestion (all notes reviewed). He was recommended to use Flonase and follow up in Rhinology clinic. He returned to the Emergency Department in August (due to scheduling errors) at which time a Sinus CT was performed. This showed mild mucosal thickening throughout the paranasal sinuses, most prominent in the right maxillary sinus where secretions were also present. He tells me that for about eight months he has had issues with his nose and sinuses. Prior to this he did have frequent sinus infections (about six times per year) and ear blockage over the ten years prior. He also had two septoplasty procedures performed for nasal fractures during this time. He started using Navage at the onset of his symptoms. He adds hydrogen peroxide to Navage. He is also using topical decongestant sprays frequently. He has had improvement with this and has been able to clear out a lot of thick and sticky mucus from his sinuses. Sometimes it is yellow-green and other times it is bloody. Since starting Navage he can breathe better through his

nose but he does have alternating congestion at night. He uses oxymetazoline spray daily for this. He reports fluctuating sense of smell. He denies sinus pressure. He also feels like his ears are clogged. He reports popping and clicking at times. He feels he has thick mucus trapped in the back of his nose and sinuses that the oxymetazoline spray and Navage cannot reach. He has no known history of seasonal allergies or asthma. He did have a Sinus CT in the ER earlier this week which showed mild mucosal thickening in the paranasal sinuses, most prominent in the right maxillary sinus where secretions were also noted (Dr. Gray interpretation).

Environmental Allergies: No

Allergy Testing: No

Asthma: No

ASA/NSAID Sensitivity: No

Prior Sinus or Nasal Surgery: Septoplasty x 2 (over ten years ago) for her history of nasal fracture

Medications Used for Nose and/or Sinuses: Navage, Oxymetazoline QD

Radiology Findings: Sinus CT (8/11/2025) - Mild mucosal thickening throughout the paranasal sinuses, most prominent in the right maxillary sinus where secretions are also present (Dr. Gray interpretation).

Past Medical History: Otherwise healthy.

Past Surgical History: Septoplasty x 2 (over ten years ago).

Medications: Klonopin, Oxymetazoline QD.

Medication Allergies: NKDA.

Social History: Never smoker.

Family History: Non-contributory.

Review of Systems: Negative other than as previously stated.

I, Elizabeth Passemato PA-C, saw the patient and performed the above elements of the history.

Physical Examination:

General appearance is well, with no signs of distress. Facial function is intact and symmetric. Eyes: No periorbital edema, extraocular movements

intact. Ears: External ears are healthy. Anterior Rhinoscopy: No lesions noted. See nasal endoscopy for full details. Oral Cavity and Oropharynx: No erythema, no lesions, palate is symmetric, tongue is midline and fully mobile, no discolored drainage from the nasopharynx. Respiratory: Breathing comfortably, with no stertor or stridor. Skin: No vascular lesions or rashes on the face or the neck. Neurologic: CN3-7 grossly intact. Psychiatric: Appropriate affect. Lymphatic: No lymphadenopathy.

As a result of the patient's sinonasal symptoms which could not be adequately evaluated by external exam or anterior rhinoscopy, the decision was made to perform a sinonasal endoscopy. We discussed the risks, benefits, options, and complications of this procedure. We discussed the risks of bleeding, pain, infection, limited views, and failure to meet the goals of the procedure.

Procedure:

After obtaining verbal consent, the nasal cavity was topically anesthetized using a combination of Lidocaine 2% and Oxmetazoline 0.025%. A zero degree endoscope attached to an HD camera was then introduced into bilateral nasal cavities. The interior of bilateral nasal cavities, middle and superior meati, the turbinates, and the spheno-ethmoidal recesses were examined. The video feed was projected onto the wall monitor in front of the patient and available for the patient to follow along.

Nasal cavity: More edema of the right nasal passage. Inflammation of the septum and lateral wall bilaterally.

Inferior turbinates: Edematous bilaterally, more on the right. Clear mucus draping posteriorly bilaterally.

Middle turbinates and middle meatus: The middle turbinates are minimally edematous. No mucus or inflammation in the middle meatus visible.

Superior turbinates and spheno-ethmoid recess: No mucus or inflammation.

Nasopharynx: Foamy secretions.

Impression:

1. Rhinitis medicamentosa.
2. Chronic rhinosinusitis.

Plan:

Mr. May and I reviewed his history and symptoms and the findings on nasal endoscopy and prior imaging. First we reviewed the diagnosis of rhinitis medicamentosa. I explained that he needs to stop using the Oxymetazoline spray. I also asked him to stop using the Navage and

hydrogen peroxide in the irrigations as this is causing inflammation. We then reviewed the diagnosis of chronic rhinosinusitis. CRS is an inflammatory condition of the paranasal sinuses that may cause a substantial quality of life detriment, and cause symptoms of nasal congestion, mucus production, diminished sense of smell, and facial pressure. Long-term medical management directed at reducing sinonasal inflammation and enhancing mucus clearance is the mainstay of treatment. As initial treatment, I would recommend a prednisone taper (this will also help with Oxymetazoline cessation). We discussed the risks including GERD, ulcer, change in sleep/mood, worsening HTN, glaucoma/cataracts, osteoporosis, avascular necrosis of the hip. I would also like him to start dilute budesonide irrigations. A prescription and instructions was provided. If appropriate medical management is not sufficient to improve symptoms and patient quality of life, surgical intervention can be considered. Surgical intervention is not intended to be curative for CRS, but rather changes the sinonasal anatomy and facilitates topical directed sinonasal therapy (with interventions such as nasal irrigations). Long-term medical management is typically required even after surgical intervention. I asked him to return in 4-6 weeks for repeat examination. Repeat imaging might be necessary. He will also proceed with Otology evaluation for his ear symptoms. All questions and concerns were answered.

Medication Review:

The risks, benefits, and side effects of prescription drug management with topical steroid irrigations were reviewed and/or made available to the patient including the fact that they are not FDA approved and are being prescribed off-label. As such they may carry the same potential risks of oral steroids including Cataracts, Hypertension, Fluid Retention, High Blood Sugar, Diabetes, Osteoporosis, Gastritis, Avascular Necrosis, Immunosuppression, Skin Changes, Weight Gain, Adrenal Gland Suppression. The patient was counseled on the need for surveillance of these potential side effects including the need for prophylactic eye exams while on topical steroids.

I personally saw and evaluated the patient with Elizabeth Passemato, PA-C. I performed the history, examination, nasal endoscopy and impression and plan myself. I personally made all medical and surgical diagnoses and decisions for the patient. I have reviewed the note in its entirety and composed/edited before signing.

Stacey T Gray, MD

Division of Rhinology

Department of Otolaryngology
Massachusetts Eye and Ear Infirmary

CC:

Pcp, Not Required

Unknown, Unknown, MD